

Patient Results

F

Path Report

Final

Final

SURGICAL PATHOLOGY REPORT

ACCESSION NO. : [REDACTED]

Final Diagnosis(es):

(A) Left simple mastectomy:

- 1) Benign breast with areas of stromal fibrosis.
- 2) No atypia, in-situ or invasive malignancy identified, including margins.
- 3) Skin and nipple unremarkable.

(B) Right simple mastectomy:

- 1) Infiltrating lobular carcinoma, Nottingham grade II of III.
- 2) Pathologic stage pT3 pN0 pMX
- 3) See Synoptic report (below) for details.

(C) Right axilla sentinel lymph node #1: One lymph node negative for metastasis (0/1).

Synoptic Report:

Specimen Type: Total breast (including nipple and skin).

Procedure: Simple mastectomy.

Lymph Node Sampling: Sentinel lymph node.

Laterality: Right.

Tumor Type: Lobular.

Tumor Focality: Single.

Tumor Site: Lower outer quadrant.

Tumor Size: 7.8 x 7.0 x 6.0 cm.

Nottingham Grade: II of III.

Skin: Present, unremarkable.

Nipple: Present, unremarkable.

Skeletal Muscle: Absent.

Invasive Margins: Widely negative.

Lobular Carcinoma in Situ: Not present.

Ductal Carcinoma in Situ: Not present.

Lymphovascular Invasion: Not identified.

Lymph nodes: Sentinel: 0/1 (positive/total)

CK 10.11 performed on block B1.

Ancillary Studies:

Estrogen Receptor: Positive, strong, 95%

Progesterone Receptor: Positive, strong, 50%

Her-2/neu: 0+ (0-3+ scale)

(was not sent for FISH analysis)

KI-67: 25%

MD

Specimen(s) Received:

A: left simple mastectomy

B: right simple mastectomy

C: right axilla sentinel lymph node # 1

ICD-0-3

carcinoma, infiltrating lobular, NOS 8520/3

Site: breast, NOS C50.9

hw
11/4/12

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Clinical History:

High risk breast cancer left.

(A) Short - superior, long - lateral.

(B) Short superior, long - lateral.

(C) Count 166

Intraoperative Consultation:

Time Received:

Time Reported:

FSC1-FSC2: Right axilla sentinel lymph node #1: Benign (2 blocks).

M.D.

Gross Description:

(A) (left simple mastectomy)

Specimen type: Simple mastectomy.

Specimen weight and dimensions: The specimen weighs 1595 gm and measures 23.5 x 22.5 x 6.3 cm.

Skin: The overlying skin ellipse measures 22.0 x 11.0 cm. The skin appears tan-pink and unremarkable.

Orientation and inking scheme: The specimen is oriented as follows: One short suture denoting the superior orientation and one long suture representing the lateral left orientation. The specimen is inked as follows: Medial - red, lateral - green.

Estimated size of lesion: Sectioning of the breast demonstrates unremarkable fibrofatty tissue with no nodules or masses seen or palpated.

Blocks submitted:

A1 - section of nipple

A2-A3 - two sections from upper lateral portion of breast

A4-A5 - two representative sections of the lower lateral quadrant

A6-A7 - representative sections of lower medial quadrant

A8-A9 - representative sections of upper medial quadrant

(B) (right simple mastectomy)

Specimen type: Simple mastectomy.

Specimen weight and dimensions: The specimen weighs 1670 gm and measures 26.0 x 28.0 x 6.0 cm.

Skin: The ellipse of overlying skin measure 24.5 x 9.0 cm. The skin is pink and unremarkable.

Orientation and inking scheme: The specimen is oriented as follows: One short suture denoting the superior orientation and one long suture representing the lateral left orientation. The specimen is inked as follows: Deep margin - black, anterior lateral margin - red, anterior medial margin - blue.

Estimated size of lesion: The breast is serially sectioned. The primary lesion measures approximately 7.0 x 7.8 x 6.0 cm.

Appearance of lesion: The lesion is firm and tan-white, and infiltrates into the breast parenchyma.

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Location of lesion: The lesion is located approximately 7.0 cm medial to the center of the specimen in the lower lateral quadrant.

Distance from closest margin: The primary lesion is located 1.5 cm superficial to the deep margin, 4.0 cm from the anterior margin, 6.0 cm from the lateral margin, 19.0 cm to the medial margin, 6.0 cm to the inferior margin, and 14.0 cm to the superior margin.

Other findings: Two possible satellite lesions are located with one central and one at 2:00. The central lesion measures 6.5 x 3.0 x 3.0 cm. The lesion at 2:00 measures 1.5 x 2.5 x 2.5 cm.

Uninvolved parenchyma: The uninvolved parenchyma demonstrates unremarkable fibrofatty tissue.

Axillary tail and lymph nodes: No axillary tail is received in this specimen.

Blocks submitted:

B1 - nipple

B2 - fat overlying tumor

B3 - skin overlying tumor

B4-B8 - representative sections from the primary tumor

B9-B10 - representative sections from the centrally located possible satellite lesion

B11-B12 - two representative sections from the lesion at 2:00

B13-14 - representative sections of uninvolved parenchyma from the upper medial quadrant

B15-B16 - representative sections of uninvolved parenchyma

B17-B18 - representative sections of the lower lateral quadrant

B19-B20 - two representative sections of uninvolved parenchyma from the upper lateral quadrant

B21 - section of the medial margin

B22 - section of the lateral margin

B23 - section of the inferior margin

B24 - section of the superior margin of uninvolved parenchyma

(C) (right axilla sentinel lymph node #1) Received fresh for intraoperative consultation is one piece of fatty tissue measuring 3.3 x 2.4 x 1.0 cm. The specimen is bisected with one-half submitted in FSC1 and the other half submitted in FSC2 for frozen section evaluation. One lymph node was received in formalin and submitted entirely in C3. [REDACTED]

Microscopic Description:

Complete microscopic evaluation has been performed. [REDACTED]

Appropriately reacting controls have been performed and evaluated for all stains on this case as required.

Histopathology has a list of IH antibodies that are regulated as analyte specific reagents (ASR's). These assays were developed and their performance characteristics determined by the

[REDACTED] at [REDACTED] They have not been cleared by the US Food and Drug Administration.

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The FDA has determined that such clearance or approval is not necessary for the ASR's. These tests are not investigational and are used in standard clinical care. In cases where immunohistochemistry testing is performed, the following antibodies and their respective clones may be used to determine therapy for the patient: EFGR(31G7), ER(SP1), PR(1E2), Her2neu(4B5), CD117(Poly), CD20(L26). Unless otherwise stated in the report, all tissue tested for ERPR by IHC, Her2 by IHC and/or HER2 by FISH have been fixed as per ANP.22998 for a minimum of 6 hours and a maximum of 48 hours.

ER, PR, Ki-67, p53 are reported as a semi-quantitative percentage of positively stained nuclei. Her-2/neu and EGFR are scored as follows: No staining at all is scored as (0), weak, incomplete membrane staining in any proportion of cells is scored as (1+), less than strong but complete staining in any proportion of cells or complete strong staining in less than 30% of cells is scored as (2+), and strong complete staining in more than 30% of cells is scored as (3+). All studies are performed on tissue fixed in 10% neutral buffered formalin and embedded in paraffin unless otherwise stated in the report.

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
Staging Discrepancy		X
For Malignancy History		X
Consistency: Primary Notes		X
Case to Case		X
Reviewed	QUALIFIED	DISQUALIFIED
	11/16/12	11/16/12

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